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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 30 of 117



A 37-year-old African American female presents with shortness of breath with minimal exertion. She also notes weight gain and persistent swelling in her legs after having twins 2 months prior. She complains of bilateral chest discomfort while breastfeeding. She has a history of hypertension preceding pregnancy, gestational diabetes, and obesity. Medications include labetalol and a multivitamin. BP is 132/85, HR 109 bpm, BMI is 33, JVP 13 cm H₂O, chest tender to palpation, abdomen obese, and 1+ bilateral edema on exam. ECG demonstrates sinus tachycardia. Echocardiogram reveals severely reduced biventricular function with LVEF 18% and large apical thrombus.

In addition to the labetalol she is currently taking, which of the following medications is considered safe for use in lactating women?

A. Sacubitril valsartan



B. Furosemide

C. Bromocriptine

D. Empagliflozin

E. Apixaban

Commentary

References

Answer: B. Furosemide

While treatment for HFrEF is well established, the evidence about the safety of medicines passed to infants during breastfeeding is scarce and largely of poor quality; this often leads to incorrect advice that mothers diagnosed with peripartum cardiomyopathy should not breastfeed. Given its benefits to both mother and infant, breastfeeding should not routinely be ruled out if the mother is taking heart failure medication but the consequences for the mother and infant need to be considered. An informed risk assessment to minimize potential harm to the infant can be carried out using the evidence that is available along with a consideration of drug properties, adverse effects, pediatric use and pharmacokinetics. In most cases, risks can be managed and infants can be monitored for

potential problems, when the mother is well enough. Breastfeeding can be encouraged in women with cardiac dysfunction with PPCM although treatment for the mother takes priority with breastfeeding compatibility being the secondary consideration. Recent data from IPAC demonstrated that breastfeeding was not associated with adverse cardiac outcomes, inflammatory markers, or persistent myocardial dysfunction in women with cardiomyopathies. Many HF medications can be given safely with breastfeeding (Figure 1) and should not be a reason to advise women against lactation. Furosemide can be safely used in lactating women to control volume overload, however care must be taken to avoid overdiuresis, which is associated with a decrease in milk production.

Answer A: Insufficient data exists for use of sacubitril-valsartan in lactating mothers with cardiomyopathy. Neonates may be highly sensitive to angiotensin blocking drugs, which are secreted into breast milk in small amounts, although captopril and enalapril are considered safe in lactating mothers. When concerns exist, hydralazine does not pass in substantial amounts into breast milk and can be safely used.

Answer C: Bromocriptine has been shown to be associated with myocardial recovery in one study of peripartum cardiomyopathy. Its original use however was to stop lactation by drying up milk supply, so it should not be used in mothers who wish to continue lactation.

Answer D: SGLT2 inhibitors pass into the breast milk and are new enough that no data exist on their effects on the newborn. Current advice is that they should be avoided.

Answer E: Warfarin and low molecular weight heparin are safe in lactation, but insufficient data exist on the newer oral anticoagulants and it is generally recommended that they be avoided while breast feeding.

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